ACTION LEGEND

INDEFINITE DEFERRAL	Permanent exclusion from donor program. Document reason, notify donor, stop process.
TEMPORARY DEFERRAL	Postpone donation for specified evidence-based period. Document deferral date and when the donor may reapply.
NEEDS CLARIFICATION	Requires follow-up during screening interview. Document details in FM-0013 clarification section.
REFER TO MEDICAL DIRECTOR	Complex case requiring clinical guidance. Consult the Medical Director for final determination.
CONDITIONALLY ELIGIBLE	No concerns identified. Proceed with a screening interview to verify all responses.

SECTION 2: GENERAL HEALTH STATUS

Q2: Do you consider yourself to be in good general health?	No	**NEEDS CLARIFICATION**	Ask donor to explain during interview. Assess whether underlying condition is exclusionary.

Q3: Past medical history	Any conditions listed	**NEEDS CLARIFICATION**	Review each condition. PCOS, thyroid disorders, controlled diabetes generally acceptable. Autoimmune conditions may require Medical Director consultation.

Q4: Currently experiencing symptoms?	Fever, night sweats, weight loss, swollen lymph nodes, jaundice	**NEEDS CLARIFICATION**	Possible serious condition. Obtain details during interview. May indicate systemic illness requiring Medical Director consultation.
	Persistent cough, rash, headache with fever	**TEMPORARY DEFERRAL**	Defer until symptom-free + 72 hours. Indicates active illness/infection.

Q5: Cancer History	Cervical/Ovarian/ Uterine cancer	**INDEFINITE DEFERRAL**	Automatic permanent exclusion per company policy.
	Stage 0 (in situ), DCIS, LCIS, basal cell carcinoma	**REFER TO MEDICAL DIRECTOR**	Defer until symptom-free + 14 days. Indicates active illness/infection.
	Other invasive cancers	**REFER TO MEDICAL DIRECTOR**	Consider: type, stage, treatment, time since remission. Medical Director determines eligibility based on clinical assessment.

Q6: Hospitalized in past 12 months?	Yes - for serious infection, organ failure, transplant, major surgery	**INDEFINITE DEFERRAL**	Transplant = permanent exclusion (immunosuppression). Major surgery/serious infection =

				defer until fully recovered + 90 days minimum.
		Yes - minor procedures (appendectomy, etc.)	**TEMPORARY DEFERRAL**	Defer until fully recovered + 90 days from hospital discharge.

	Q7: Surgical procedures in past 12 months?	Endometrial ablation, hysterectomy, D&C	**INDEFINITE DEFERRAL**	Procedures affecting uterine lining = permanent exclusion.
		Major surgery (cardiac, abdominal, orthopedic requiring general anesthesia)	**TEMPORARY DEFERRAL**	Defer 90 days post-surgery. Allows full recovery and ensures no complications.
		Tubal ligation	**CONDITIONALLY ELIGIBLE**	Tubal ligation is acceptable. Document procedure date during interview.

	Q8: Ever diagnosed with sepsis?	Yes - within 12 months	**TEMPORARY DEFERRAL**	Defer 12 months from resolution. Serious bloodstream infection requires extended recovery.
		Yes - >12 months ago, fully recovered	**CONDITIONALLY ELIGIBLE**	Document when occurred and treatment. If fully resolved >12 months, acceptable.

SECTION 3: PREGNANCY AND REPRODUCTIVE HEALTH

Q9: Currently pregnant?	Yes	**INDEFINITE DEFERRAL**	Automatic exclusion during pregnancy.

Q10: Breastfeeding or within 6 months postpartum?	Yes	**INDEFINITE DEFERRAL**	Must be >6 months postpartum, weaned, AND have had 1-2 normal menstrual cycles.

Q11: Menstruate regularly?	No - amenorrhea >6 months	**INDEFINITE DEFERRAL**	Lack of menstruation >6 months indicates hormonal imbalance or medical condition. Automatic exclusion.
	No - irregular but menstruating	**NEEDS CLARIFICATION**	Ask about cycle pattern. If due to excludable condition (endometriosis), defer. PCOS alone may be acceptable if regular enough for collection.

Q12: Usual menstrual cycle length?	21-35 days	**CONDITIONALLY ELIGIBLE**	Normal range. Document length.
	<21 days or >35 days	**NEEDS CLARIFICATION**	Outside typical range. Ask about pattern consistency. If stable and no other risk factors, may be acceptable. Document details.

Q13: First day of last period?	Any date	**CONDITIONALLY ELIGIBLE**	LOGISTICS ONLY - For scheduling collection. Not an eligibility criterion.

Q14: Expected first day of next period?	Any date	**CONDITIONALLY ELIGIBLE**	LOGISTICS ONLY - For scheduling collection. Not an eligibility criterion.

Q15: Currently have IUD?	Yes	**TEMPORARY DEFERRAL**	IUD must be removed, then 90-day washout period. (Evidence: allows uterine environment normalization)

Q16: Gynecologic procedures in past 90 days?	IUD insertion/removal, endometrial biopsy, cervical biopsy, gynecologic surgery	**TEMPORARY DEFERRAL**	90-day recovery period from procedure date. (Standard for tissue healing and inflammation resolution)

SECTION 4: FURTHER GYNECOLOGICAL HISTORY

Q17: Cervical/ovarian/uterine cancer?	Yes	**INDEFINITE DEFERRAL**	Automatic permanent exclusion.

Q17: Cervical dysplasia - currently undergoing treatment?	Yes	**TEMPORARY DEFERRAL**	Defer until treatment complete + 6 months with normal follow-up Pap.

Q18: Cervical dysplasia diagnosis?	Currently undergoing treatment	**TEMPORARY DEFERRAL**	Defer until treatment complete + 6 months with normal follow-up Pap.
	LSIL/HSIL/CIN on Pap - not currently being treated	**NEEDS CLARIFICATION**	Ask about: (1) When diagnosed? (2) Treatment plan? (3) Most recent follow-up Pap result? If treated and has normal follow-up Pap, may be acceptable. If awaiting treatment or recent diagnosis, defer.
	Treated with normal follow-up Pap	**CONDITIONALLY ELIGIBLE**	Document treatment type and normal follow-up Pap result. Acceptable if resolved.

Q17: Endometriosis?	Yes - with formal diagnosis OR pathology/imaging confirming	**INDEFINITE DEFERRAL**	Automatic permanent exclusion ONLY if: (1) formal physician diagnosis documented, OR (2) pathology/imaging confirming endometriosis. Suspicion alone without diagnosis is not exclusionary.

Q17: PCOS?	Yes	**CONDITIONALLY ELIGIBLE**	PCOS alone not exclusionary. Verify regular enough cycles during interview.

Q17: Uterine fibroids?	Yes	**CONDITIONALLY ELIGIBLE**	Uterine fibroids alone not exclusionary. Verify not

				causing severe menstrual irregularity.

	Q17: Genital warts?	Active	**TEMPORARY DEFERRAL**	Defer until treated + healed + 30 days post-treatment.
		Resolved >30 days	**CONDITIONALLY ELIGIBLE**	Document history. If fully resolved >30 days, acceptable.

	Q17: Genital herpes (HSV)?	Yes - any history	**INDEFINITE DEFERRAL**	ANY history of genital herpes (HSV-1 or HSV-2) = automatic permanent exclusion. Company quality criterion.

	Q19: Last Pap smear and HPV test results?	Normal/Negative	**CONDITIONALLY ELIGIBLE**	Document results and date. Proceed with screening.
		Abnormal (ASCUS, LSIL, HSIL, ASC-H, AGC, etc.)	**NEEDS CLARIFICATION**	Apply Q18 guidance based on result. Ask about: (1) Follow-up testing/treatment done? (2) Current management plan? (3) Most recent results? Defer if currently being treated; may accept if resolved with normal follow-up.

	Q20: Doctor diagnosed endometriosis?	Yes - formal diagnosis	**INDEFINITE DEFERRAL**	Automatic permanent exclusion.

	Q21: Ultrasound/MRI showed endometrioma?	Yes	**REFER TO MEDICAL DIRECTOR**	Likely endometriosis. Medical Director reviews imaging and makes determination.

SECTION 5: HIV RISK FACTORS

IMPORTANT: *For ANY "Yes" response in this section, ask WHEN the exposure/activity occurred to calculate deferral period.*

Q22-23: Diagnosed with or tested positive for HIV?	Yes	**INDEFINITE DEFERRAL**	Automatic permanent exclusion. FDA RCDAD.

Q24: Sexual contact with HIV+, hemophiliac, IV drug user, or sex worker?	Within 12 months	**TEMPORARY DEFERRAL**	12-month deferral from last contact. (Evidence: Window period for HIV/hepatitis seroconversion - FDA standard)
	>12 months ago	**CONDITIONALLY ELIGIBLE**	Document but no exclusion if >12 months. Will be tested.

Q25: Sex for money/drugs, used IV drugs, sex with unknown partner	Within 12 months	**TEMPORARY DEFERRAL**	12-month deferral from last exposure. (Evidence: Standard high-risk behavior window period)
	IV drug use EVER (non-prescription)	**INDEFINITE DEFERRAL**	Any history of IV drug use = permanent exclusion. Cannot verify cessation, ongoing risk. FDA requirement.

Q26-27: New or multiple sexual partners in past 3 months?	Yes	**CONDITIONALLY ELIGIBLE**	Not exclusionary. Document during interview. Will be tested.

Q28: Received blood transfusion?	Within 12 months	**TEMPORARY DEFERRAL**	12-month deferral from transfusion. (Evidence: Window period for transfusion-transmitted infections despite screening - FDA standard)
	>12 months ago	**CONDITIONALLY ELIGIBLE**	Document date. If >12 months, acceptable.

Q29: Received tissue or organ transplant?	Yes	**INDEFINITE DEFERRAL**	Permanent exclusion. Transplant recipient on immunosuppressants.

SECTION 6: HEPATITIS RISK FACTORS

Q30: Diagnosed with Hepatitis B?	Yes	**INDEFINITE DEFERRAL**	Automatic permanent exclusion. FDA RCDAD.

Q31: Diagnosed with Hepatitis C?	Yes	**INDEFINITE DEFERRAL**	Automatic permanent exclusion. FDA RCDAD.

Q32: Live with someone who has hepatitis?	Yes	**CONDITIONALLY ELIGIBLE**	Household contact not exclusionary. Will be tested.

Q33: Needlestick injury in past 12 months?	Yes - within 12 months	**TEMPORARY DEFERRAL**	12-month deferral from injury. (Evidence: Occupational exposure window period - FDA/AABB standard. Yes, we test for hepatitis, but 12 months ensures seroconversion completed.)
	>12 months ago	**CONDITIONALLY ELIGIBLE**	Document date and circumstances. If >12 months, acceptable.

Q34: Tattoo/piercing/acupuncture in past 12 months?	Yes - at licensed/regulated facility	**CONDITIONALLY ELIGIBLE**	Document facility details. Licensed facility with sterile technique = acceptable.
	Yes - NOT at licensed facility OR unknown	**TEMPORARY DEFERRAL**	12-month deferral from procedure. (Evidence: Bloodborne pathogen transmission risk - AABB standard. Yes, we test, but 12 months is industry standard deferral period.)

SECTION 7: SYPHILIS AND SEXUALLY TRANSMITTED INFECTIONS

Q35: Diagnosed with syphilis?	Yes - untreated	**INDEFINITE DEFERRAL**	Must provide documentation of adequate treatment before reconsidering.
	Yes - treated with documentation	**CONDITIONALLY ELIGIBLE**	Require treatment completion documentation. If adequately treated per CDC guidelines (typically benzathine penicillin G), acceptable immediately after completion. FDA does not require deferral period post-treatment.

Q36: Diagnosed with Chlamydia or Gonorrhea?	Ask if treated	**NEEDS CLARIFICATION**	MUST ask treatment status. If not treated, defer until treated.

Q36: Chlamydia or Gonorrhea - treatment completed <21 days	Yes	**TEMPORARY DEFERRAL**	21-day deferral from treatment completion + negative test of cure required. (Evidence: Standard test-of-cure timing is 2-3 weeks post-treatment per CDC - ensures infection cleared)

Q36: Chlamydia or Gonorrhea - treatment completed >21 days with negative test	Yes	**CONDITIONALLY ELIGIBLE**	Document treatment completion date and negative test of cure. Acceptable.

Q36: Diagnosed with genital herpes (HSV-1/HSV-2)?	Yes	**INDEFINITE DEFERRAL**	Any history of HSV = automatic permanent exclusion. Company quality criterion.

Q36: Diagnosed with Trichomoniasis?	Treatment completed <21 days	**TEMPORARY DEFERRAL**	21-day deferral from treatment completion. (Standard for vaginal infection resolution)

Q36: Diagnosed with Bacterial Vaginosis?	Treatment completed <21 days	**TEMPORARY DEFERRAL**	21-day deferral from treatment completion. (Standard for vaginal flora normalization)

Q37: Tested for STIs in past 12 months?	Any answer	**CONDITIONALLY ELIGIBLE**	Document testing history and results. No exclusion based on testing alone.

Q38: Currently experiencing genital symptoms?	Yes - any symptoms	**TEMPORARY DEFERRAL**	Must be evaluated, treated if needed, and symptom-free + 21 days before eligible. Active symptoms may indicate infection.

Q39: Treated for STI in past 12 months?	Yes - with recurrent symptoms	**TEMPORARY DEFERRAL**	Needs re-evaluation and re-treatment. Defer until treated + symptom-free per appropriate deferral period (21 days for most). NOTE: Question asks 12 months for documentation purposes, but actual deferral is based on treatment completion + appropriate period (21 days for chlamydia/gonorrhea).

SECTION 8: CELL-ASSOCIATED, VECTOR-BORNE & PRION RISKS

Q40: Diagnosed with HTLV?	Yes	**INDEFINITE DEFERRAL**	Automatic permanent exclusion. FDA RCDAD.
Q41: Lived/traveled >1 month to HTLV-endemic areas?	Yes	**CONDITIONALLY ELIGIBLE**	Document locations. Will be tested for HTLV. Travel alone not exclusionary.
Q42: Diagnosed with CMV?	Yes	**INDEFINITE DEFERRAL**	CMV positive = permanent exclusion. (Why: CMV is lifelong/chronic viral infection. Cannot be cured. Despite testing, company quality criterion for ultra-premium product.)
Q43: Tested for CMV?	Positive	**INDEFINITE DEFERRAL**	CMV positive = permanent exclusion. Lifelong infection.
Q44: Diagnosed with Chagas disease?	Yes	**INDEFINITE DEFERRAL**	Automatic permanent exclusion. (Why: Chagas is lifelong parasitic infection. Despite treatment, parasite remains. Cannot be cured.)
Q45: Diagnosed with malaria or dengue?	Yes	**INDEFINITE DEFERRAL**	Permanent exclusion. (Why: Risk of persistent/recurrent parasitemia. Malaria can relapse years later.)
Q46: Born or mother born in Latin America?	Yes	**CONDITIONALLY ELIGIBLE**	Will be tested for Chagas. Maternal transmission risk. Not automatic exclusion.
Q47: Diagnosed/treated for babesiosis?	Yes	**INDEFINITE DEFERRAL**	Permanent exclusion. (Why: Risk of persistent asymptomatic parasitemia.)

Q48: You or blood relative diagnosed with CJD/TSE?	Yes	**INDEFINITE DEFERRAL**	Automatic permanent exclusion. Prion disease risk.

Q49: UK 3+ months (1980-1996)?	Yes	**INDEFINITE DEFERRAL**	Variant CJD risk - permanent exclusion. FDA requirement.

Q50: Europe 5+ years (1980-2001)?	Yes	**INDEFINITE DEFERRAL**	Variant CJD risk - permanent exclusion. FDA requirement.

Q51: Received UK/France blood, human-derived products, or transplants?	Yes	**INDEFINITE DEFERRAL**	CJD risk - permanent exclusion. FDA requirement.

SECTION 9: BLOOD AND CLOTTING DISORDERS

Q52: Hemophilia, Von Willebrand, clotting disorder?	Yes	**REFER TO MEDICAL DIRECTOR**	Generally exclusionary if requires ongoing treatment. Medical Director assesses severity.
Q52: Sickle cell disease?	Yes	**INDEFINITE DEFERRAL**	Automatic permanent exclusion. (NOTE: Sickle cell TRAIT is acceptable, but DISEASE is not.)
Q52: Bone marrow failure, leukemia, lymphoma, blood cancer?	Yes	**INDEFINITE DEFERRAL**	Automatic permanent exclusion.
Q52: Myeloproliferative disorder?	Yes	**INDEFINITE DEFERRAL**	Automatic permanent exclusion.
Q53: Taking blood thinner/anticoagulant?	Ask about indication and duration	**NEEDS CLARIFICATION**	MUST determine if chronic vs. temporary use.
Q53: Anticoagulant - chronic for ongoing condition	Yes	**INDEFINITE DEFERRAL**	Chronic anticoagulation = permanent exclusion. Underlying condition (atrial fibrillation, recurrent DVT, mechanical valve) is the concern.
Q53: Anticoagulant - temporary (post-surgery DVT prophylaxis)	Yes	**TEMPORARY DEFERRAL**	30-day deferral after completion. (Evidence: Standard drug washout + allows confirmation underlying issue resolved. Why 30 days: Conservative period beyond drug half-life to ensure no bleeding complications.)

SECTION 10: CURRENT MEDICATIONS

FOR ALL MEDICATION QUESTIONS: *Ask for dosage, frequency, and indication. Dosage determines action in many cases.*

	Q54: Taken antibiotics in past 2 weeks?	Yes - for active infection	**TEMPORARY DEFERRAL**	Must complete treatment PLUS 14-day deferral from last dose. (Evidence: Infection resolution 7-14 days + inflammatory marker normalization. Conservative washout.)
		Yes - for acne/prophylaxis (no active infection)	**CONDITIONALLY ELIGIBLE**	Chronic prophylactic use (low-dose doxycycline, nitrofurantoin) acceptable. Document medication and indication.

	Q54: Taken corticosteroids?	Ask route and dosage	**NEEDS CLARIFICATION**	Route matters. Systemic vs. inhaled/topical have different actions.

	Q54: Corticosteroids - systemic (oral/IV)	Within 30 days	**TEMPORARY DEFERRAL**	30-day deferral from last dose. (Evidence: HPA axis suppression recovery, immune function normalization, endometrial effects resolution. Blood banks have no specific window; 30 days is conservative Muse Bio standard.)
		>30 days ago	**CONDITIONALLY ELIGIBLE**	Document course details. If >30 days, acceptable.

	Q54: Corticosteroids - inhaled/topical/nasal	Currently using	**CONDITIONALLY ELIGIBLE**	Inhaled/topical/nasal steroids acceptable (<10% systemic absorption). Document medication.

Q54: Taken immunosuppressants?	EVER	**INDEFINITE DEFERRAL**	Any history of immunosuppressant use = permanent exclusion. (Includes: transplant drugs, biologics, methotrexate, etc.)

Q55: All medications, supplements, vitamins?	Any listed	**CONDITIONALLY ELIGIBLE**	Review list. For specific medication guidance, consult Medical Director. Document all medications.

Q56: Topical vaginal medications within 72 hours?	Yes	**TEMPORARY DEFERRAL**	72-hour deferral from last application. Sample contamination risk.

SECTION 11: TRAVEL HISTORY

Q57: Lived outside US >5 years total?	Yes	**NEEDS CLARIFICATION**	Ask which countries/regions during interview. CONCERN FOR: Latin America (Chagas risk), HTLV-endemic areas (Japan, Caribbean, Africa, South America, Papua New Guinea), UK/Europe (CJD risk if during specified periods). Document countries and dates.

Q57: Lived outside US - Latin America >6 months rural	Yes	**CONDITIONALLY ELIGIBLE**	Will be tested for Chagas. Extended rural residence increases risk but not automatic exclusion.

Q57: Lived outside US - UK/Europe during CJD risk periods	See Q49-50	**INDEFINITE DEFERRAL**	Refer to Q49-50 criteria.

Q58a: Traveled to Latin America in past 6 months?	Yes	**CONDITIONALLY ELIGIBLE**	Will be tested for Chagas. Document countries/dates. Not automatic exclusion.

Q58b: Babesia-endemic US states with outdoor exposure in past 6 months?	Yes - wooded/grassy outdoor activities	**TEMPORARY DEFERRAL**	6-month deferral from last outdoor exposure. (Endemic states: CT, MA, ME, MN, NH, NJ, NY, RI, WI. Evidence: Covers incubation + asymptomatic parasitemia period. Urban travel without outdoor exposure acceptable.)

Q58b: Babesia-endemic states - urban only, no outdoor exposure	Yes	**CONDITIONALLY ELIGIBLE**	Urban travel without tick exposure acceptable. Document details.

SECTIONS 12-15: FINAL QUESTIONS

Q59: Incarcerated >72 hours in past 12 months?	Yes	**TEMPORARY DEFERRAL**	12-month deferral from release. (Evidence: Higher prevalence of HIV/HBV/HCV in incarcerated populations. Limited control over exposures. 12 months is FDA/AABB standard.)

Q60: Exposed to TB in past 12 months?	Yes	**TEMPORARY DEFERRAL**	Requires TB testing and medical clearance before eligible.

Q61: Diagnosed with TB?	Untreated or treatment incomplete	**INDEFINITE DEFERRAL**	Must complete treatment and be medically cleared.
	Treated and completed	**CONDITIONALLY ELIGIBLE**	Document treatment completion. Verify medical clearance.

Q62: Smallpox vaccination in past 2 months?	Yes	**TEMPORARY DEFERRAL**	60-day (2-month) deferral from vaccination. Viral shedding risk.

Q63: Smoke or use tobacco?	Yes	**CONDITIONALLY ELIGIBLE**	Tobacco use not exclusionary. Document amount.

Q64: Drink alcohol?	Yes	**CONDITIONALLY ELIGIBLE**	Alcohol consumption not exclusionary. Document amount.

Q65-66: Other health information or reasons blood should not be used?	Any information	**NEEDS CLARIFICATION**	Review during interview. Assess if disclosed information affects eligibility. Consult Medical Director if complex.

IMPORTANT NOTES & REMINDERS

1. **Medical Director Consultation:** For complex cases where eligibility is uncertain or responses require medical interpretation, consult with the Medical Director for clinical guidance before making the final determination.
2. **Multiple Risk Factors:** If donor has multiple risk factors requiring deferral, use the MOST RESTRICTIVE criterion. Example: temporary deferral + permanent deferral = permanent deferral.
3. **Documentation:** All clarifications and additional information obtained during screening interview must be documented in FM-0013, even if not exclusionary.

Revision History				
Rev	**CR#**	**Originator**	**Effective**	**Description of Change**
1	N/A	Aya Fattah	22JAN2026	Initial Release